

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase III, Single-Arm Multicenter Study of Atezolizumab (Anti-PD-L1 Antibody) in High PD-L1 Expression, Chemotherapy-Naïve Patients With Stage IV Non-Squamous or Squamous Non-Small Cell Lung Cancer
Short Title:	A Study of Atezolizumab in High PD-L1 Expression, Chemotherapy-Naïve Patients With Stage IV Non-Squamous or Squamous Non-Small Cell Lung Cancer
Protocol Number:	35454881208951339
NCT Number:	NCT05047250
Sponsor Name:	Roche
Investigational Product:	atezolizumab
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Overall survival (OS): Overall survival (OS) is defined as the time from atezolizumab initiation to death from any cause.
Secondary Obj.:	1. Progression-free survival (PFS) 2. OS Rate at 1-Year 3. OS Rate at 2-Year

2.2 Background & Rationale

This is a Phase III, single arm, multicenter study designed to evaluate the efficacy and safety of atezolizumab in high PD-L1 expression, chemotherapy-naïve, without a sensitizing EGFR mutation or ALK translocation patients with stage IV non-squamous or squamous NSCLC.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: ACTIVE_NOT_RECRUITING
Target N:	60

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * ECOG performance status of 0 or 1.
- * Histologically or cytologically confirmed, Stage IV non-squamous or squamous NSCLC.
- * No prior treatment for Stage IV non-squamous or squamous NSCLC.
- * Patients who have received prior neo-adjuvant, adjuvant chemotherapy, radiotherapy, or chemo-radiotherapy with curative intent for non-metastatic disease must have experienced a treatment free interval of at least 6 months from enrollment since the last chemotherapy, radiotherapy, or chemo-radiotherapy cycle.
- * Tumor TC3 or IC3, as determined by SP142 performed by a central laboratory on previously obtained archival tumor

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tissue or tissue obtained from a biopsy at screening.

- * Measurable disease, as defined by RECIST v1.1.

- * Adequate hematologic and end-organ function.

- * Life expectancy ≥3 months.

- * For women of childbearing potential: agreement to remain abstinent or use contraception, and agreement to refrain from donating.

4.2 Exclusion Criteria

- * Known sensitizing mutation in the EGFR gene or ALK fusion oncogene.

- * Symptomatic, untreated, or actively progressing CNS metastases.

- * Spinal cord compression not definitively treated with surgery and/or radiation, or previously diagnosed and treated spinal cord compression without evidence that disease has been clinically stable for ≥2 weeks prior to enrollment.

- * Current leptomeningeal disease.

- * Uncontrolled tumor-related pain.

- * Uncontrolled pleural effusion, pericardial effusion, or ascites requiring recurrent drainage procedures.

- * Uncontrolled or symptomatic hypercalcemia.

- * Malignancies other than NSCLC within 5 years prior to enrollment, with the exception of those with a negligible risk of metastasis or death treated with expected curative outcome.

- * Pregnancy or breastfeeding, or intention of becoming pregnant during study treatment or within at least 5 months after the last dose of atezolizumab.

- * History of severe allergic, anaphylactic, or other hypersensitivity reactions to chimeric or humanized antibodies or fusion proteins.

- * Known allergy or hypersensitivity to biopharmaceuticals produced in Chinese hamster ovary cells or any component of the atezolizumab formulation.

- * Active or history of autoimmune disease or immune deficiency.

- * History of idiopathic pulmonary fibrosis, organizing pneumonia, drug-induced pneumonitis, idiopathic pneumonitis, or evidence of active pneumonitis on screening chest CT scan.

- * Positive human immunodeficiency virus (HIV) test result at screening.

- * Patients with active hepatitis B or active hepatitis C at screening.

- * Active tuberculosis.

- * Severe infections within 4 weeks prior to randomization, including, but not limited to, hospitalization for complications of infection, bacteremia, or severe pneumonia.

- * Significant cardiovascular disease.

11. Study Identifiers & Governance

Primary ID: 35454881208951339

Registry Number: NCT05047250

Sponsor: Roche

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15. Sign-off & Approval

Principal Investigator: _____ Date: _____
Clinical Operations Lead: _____ Date: _____
Lead Statistician: _____ Date: _____